

22STCV00864 22STCV00864

County: los-angeles

Division: Civil Law and Motion

Department: F46

Hearing date: 2026-06-24

Source URL: https://www.lacourt.ca.gov/tentativeRulingNet/ui/main.aspx?casetype=civil&dept_date=CHA%2CF46%2C06%2F24%2F2026

Source SHA-256: ff6cde4d82fe1b851a85496f9d384b0e23cbbd9c58a3ac5614731b7580b75e67

Case Number: 22STCV00864 Hearing Date: June 24, 2026 Dept: F46

Dept. F-46.

Hearing Date: June 24, 2026.

Case Number: 22STCV00864.

DEFENDANT PROVIDENCE HEALTH SYSTEM SOUTHERN CALIFORNIA
DBA PROVIDENCE HOLY CROSS MEDICAL CENTER'S MOTION FOR SUMMARY JUDGMENT

Motion filed on March 27, 2026.

MOVING PARTY: Defendant Providence Health System Southern
California dba Providence Holy Cross Medical Center.

RESPONDING PARTY: Plaintiff Laura Beth Denver.

NOTICE: OK.

RELIEF REQUESTED: Defendant moves for summary judgment on
Plaintiff's complaint. RULING: Denied.

BACKGROUND

On January 10, 2022, Plaintiff Laura Beth Denver filed a complaint against
Defendant Rachel B. Long, M.D. and Does 1 through 20, alleging one cause of
action for medical malpractice.

On May 13, 2024, Plaintiff filed an amendment to complaint

naming Providence Holy Cross Medical Center as Doe 3. On June 24, 2024, Defendant Providence Health System Southern California dba Providence Holy Cross Medical Center filed its answer.

Defendant filed the instant motion on March 27, 2026, supported by the declaration of Kelly L. Kiseskey, a separate statement, and an appendix of exhibits. Plaintiff opposed on April 15, 2026, supported by the declaration of Nora Hovsepien, a response to Defendant's separate statement, and a request for judicial notice. Defendant replied on June 12, 2026. Defendant concurrently filed evidentiary objections on June 12, 2026.

REQUEST FOR JUDICIAL NOTICE

Plaintiff requests judicial notice of (1) the Declaration of Howard C. Mandel, M.D., filed in opposition to Defendant Kathryn Joann Wright, M.D.'s prior motion for summary judgment, and (2) the Court's July 17, 2025 ruling denying that motion. The request is granted as to the existence, filing, and contents of those court records. (Evid. Code, § 452, subd. (d).)

EVIDENTIARY OBJECTIONS

Defendant objects to the Declaration of Howard C. Mandel, M.D. in its entirety. The objection is overruled because it does not identify specific material by exhibit, title, page, and line number, quote or set forth the objectionable material, or state grounds for each specific statement as required by California Rules of Court, rule 3.1354(b).

LEGAL STANDARD

In reviewing a motion for summary judgment, courts apply a three-step analysis: "(1) identify the issues framed by the pleadings; (2) determine whether the moving party has negated the opponent's claims; and (3) determine whether the opposition has demonstrated the existence of a triable, material factual issue." (Hinesley v. Oakshade Town Center (2005) 135 Cal.App.4th 289, 294.)

The initial burden is on the moving party to make a prima facie showing that no triable issue of material fact exists. (Scalf v. D. B. Log Homes, Inc. (2005) 128 Cal.App.4th 1510, 1519.) A defendant moving for summary judgment or summary adjudication meets that burden by showing that one

or more elements of the cause of action cannot be established, or that a complete defense exists. (Code Civ. Proc., § 437c, subd. (p)(2).) The defendant must support the motion with evidence and may not simply point to the absence of evidence. (Aguilar v. Atlantic Richfield Co. (2001) 25 Cal.4th 826, 849-850, 854 (Aguilar).)

Only if the defendant meets that initial burden does the burden shift to the plaintiff to show a triable issue of material fact. (Code Civ. Proc., § 437c, subd. (p)(2).) The Court must view the evidence in the light most favorable to the opposing party, liberally construe the opposing party's evidence, strictly scrutinize the moving party's evidence, and resolve evidentiary doubts or ambiguities in favor of the opposing party. (Aguilar, supra, 25 Cal.4th at pp. 843, 850.)

DISCUSSION

The statute of limitations for medical malpractice is the shorter of three years from the date of injury or one year from the date the plaintiff discovers, or through reasonable diligence should have discovered, the injury. (Code Civ. Proc., § 340.5.) "Discovery" occurs when the plaintiff suspects or reasonably should suspect that the injury was caused by wrongdoing. (Jolly v. Eli Lilly & Co. (1988) 44 Cal.3d 1103, 1110-1111.)

Code of Civil Procedure section 364 requires 90 days' prior notice before commencing an action based on a health care provider's professional negligence. (Code Civ. Proc., § 364, subd. (a).) If the notice is served within 90 days of the expiration of the applicable limitations period, the time to commence the action is extended 90 days from service of the notice. (Id., subd. (d); Woods v. Young (1991) 53 Cal.3d 315, 328.) Section 364 also provides that its provisions do not apply to a defendant whose name is unknown when the complaint is filed and who is identified by a fictitious name under section 474. (Code Civ. Proc., § 364, subd. (e).)

Plaintiff's hysterectomy occurred at Providence Holy Cross Medical Center on November 27, 2020. (UMF No. 1.) Plaintiff contends she discovered her injury on December 11, 2020, during emergency surgery at Olive View Medical Center, when surgeons identified sutures from the November 27, 2020 hysterectomy penetrating the sigmoid colon. (Hovsepian Decl. ¶ 8; Ex. C to Def.'s Index of Evidence, p. 1919.) Even using Plaintiff's December 11, 2020 discovery date, the untolled one-year limitations period expired on December

11, 2021. Plaintiff served Dr. Long with a section 364 notice on October 21, 2021. (UMF No. 2.) Plaintiff filed the original Complaint on January 10, 2022, against Dr. Long and Does 1 through 20. (UMF No. 3; Compl. ¶ 3.) Plaintiff added the Hospital as Doe 3 on May 13, 2024. (UMF No. 4.) Thus, the Hospital was added after the untolled one-year limitations period. The issue is whether the Doe amendment relates back to the original Complaint.

Defendant first argues Plaintiff's section 364 notice was "applicable to Dr. Long only." Defendant reasons that the notice to Dr. Long extended the limitations period only as to Dr. Long, that "the Code of Civil Procedure § 364 extension of the statute of limitations is not applicable to Doe defendants," and that Plaintiff therefore had to file the original Complaint within the untolled section 340.5 period for any later Doe amendment to relate back. The Court is not persuaded by that categorical argument.

Camarillo v. Vaage (2003) 105 Cal.App.4th 552 addressed the interaction between Code of Civil Procedure sections 364 and 474 in a legal malpractice action arising from an alleged failure to preserve a medical malpractice claim against an unknown delivery doctor. There, counsel served a section 364 notice on the known hospital and health center shortly before the one-year limitations period expired, but did not know the identity of the delivery doctor, did not serve that doctor, and did not file a shell Doe complaint. (Id. at pp. 556-558.) The Court of Appeal held that "section 364, subdivisions (d) and (e), and section 474, read together, provide a workable and common sense rule that serves to extend the applicable statute of limitations against an unknown defendant when a section 364, subdivision (a) notice is appropriately served, according to the overall statutory scheme, upon known potential defendants in the medical malpractice context." (Id. at p. 553.) The court further explained that section 364 notices can be given only to known defendants and, under those circumstances, no shell Doe complaint was required to preserve the claim against the unknown delivery doctor. (Id. at pp. 570-571.)

Accordingly, Plaintiff's failure to serve a separate section 364 notice on the Hospital does not defeat relation back by itself. The dispositive issue is whether the Hospital was properly named as Doe 3 under section 474.

Code of Civil Procedure section 474 allows a plaintiff to designate a defendant by a fictitious name if "ignorant of the name of a

defendant” and amend upon discovery. Ignorance includes ignorance of facts giving rise to a cause of action against a defendant, even if the plaintiff knows the defendant’s identity. (Fuller v. Tucker (2000) 84 Cal.App.4th 1163, 1170; General Motors Corp. v. Superior Court (1996) 48 Cal.App.4th 580, 593-594.) Where the original complaint names Doe defendants, “the relevant inquiry when the plaintiff seeks to substitute a real defendant for one sued fictitiously is what facts the plaintiff actually knew at the time the original complaint was filed.” (McOwen v. Grossman (2007) 153 Cal.App.4th 937, 942.) “If the identity of the Doe defendant is known but, at the time of the filing of the complaint the plaintiff did not know facts that would cause a reasonable person to believe that liability is probable, the requirements of section 474 are met.” (Id. at p. 943.) “Section 474 allows a plaintiff in good faith to delay suing particular persons as named defendants until he has knowledge of sufficient facts to cause a reasonable person to believe liability is probable.” (Ibid.) Reasonable diligence is relevant to accrual under section 340.5, not section 474 ignorance. (Id. at p. 944.) Ignorance must be real and not feigned. (Sherer v. Mark (1976) 64 Cal.App.3d 834, 841; Munoz v. Purdy (1979) 91 Cal.App.3d 942; Dover v. Sadowinski (1983) 147 Cal.App.3d 113, 116.)

Defendant meets its initial burden on the section 474 issue. Plaintiff knew the surgery occurred at Providence Holy Cross Medical Center. (Hovsepian Decl. ¶ 11.) Plaintiff also knew the discharge events on which Defendant relies. Plaintiff testified that the day after surgery, staff came in and out of her room. After Dr. Long said she would not be in to discharge Plaintiff, a nurse removed the vaginal packing, and Plaintiff began to bleed “a lot.” After discharge orders were entered, a CNA helped Plaintiff sit up and get to the restroom. Plaintiff testified there was “a lot of blood,” the CNA obtained a special pad, the CNA cleaned her up, and Plaintiff kept bleeding. Plaintiff asked whether the bleeding was normal and whether someone should call or ask the nurse. Plaintiff further testified that the bleeding had not stopped when she left the Hospital. (UMF Nos. 6-12.) Plaintiff later identified the Hospital’s alleged negligence in the same terms: nursing staff knew she was bleeding profusely before discharge, failed to contact Dr. Long or another physician, and discharged her in that condition. (UMF No. 13.) This evidence supports Defendant’s inference that Plaintiff already knew facts suggesting Hospital liability when the Complaint was filed.

Plaintiff raises a triable issue. Plaintiff does not dispute that she knew the bleeding and discharge occurred. She disputes the inference

that those facts gave her actual knowledge of probable Hospital liability. Counsel declares that an initial expert consultant reviewed the available records and identified Dr. Long as the sole responsible party based on improper surgical technique. (Hovsepian Decl. ¶¶ 7-9.) Counsel further declares that, although she knew the Hospital was where the surgery and overnight stay occurred, she had no information from the consultant or otherwise when filing the Complaint that Hospital nursing staff played any role in causing Plaintiff's harm. (Id. ¶ 11.) Counsel states she first learned in April 2024, after retaining Dr. Mandel, that the Hospital's nursing staff may be liable in addition to Dr. Long. (Id. ¶ 12.) Plaintiff filed the Doe amendment within one month. (Id. ¶ 13.)

Defendant argues in reply that Dr. Mandel's declaration cannot support Plaintiff because it was filed in opposition to Dr. Wright's motion and states that Mandel would address only his opinions as to the surgeons. Defendant moves only on statute of limitations grounds, not on whether Plaintiff can prove Hospital breach or causation. For purposes of section 474, the issue is when Plaintiff actually knew facts suggesting probable Hospital liability. Mandel's declaration states he was retained in April 2024 to review whether the treating physicians and nursing personnel met the standard of care, and that he then advised counsel that the nursing staff of Providence Holy Cross Medical Center breached the standard of care and collectively caused harm. (Mandel Decl. ¶¶ 2-3.) Mandel also identifies the nursing facts underlying that opinion, including the note that Plaintiff changed her peri-pad three times due to bleeding that saturated the chair, the later discharge, and the absence of evidence that the nurse called Dr. Long or that Dr. Long evaluated Plaintiff before discharge. (Id. ¶ 13.) This supports Plaintiff's inference that the known discharge events did not amount to actual knowledge of probable Hospital liability until expert review connected those events to alleged breach and causation.

That is the distinction recognized in *McOwen*. There, the plaintiff knew the defendant physician treated him and knew the injury had occurred. The Court held the relevant question was whether, when the complaint was filed, the plaintiff actually knew facts indicating the physician's probable liability, including the later expert theory that the physician should have ordered an angiogram rather than a Doppler test. (*McOwen*, *supra*, 153 Cal.App.4th at pp. 944-945.) Here, Plaintiff knew the Hospital provided postoperative care and knew she was bleeding when she was discharged. But Plaintiff submits evidence that the initial expert review attributed the injury to Dr. Long's

surgical technique, and that the later expert review first connected the nursing and discharge facts to alleged Hospital breach and causation. On this record, the Court cannot conclude as a matter of law that Plaintiff's known discharge facts gave her actual knowledge of probable Hospital liability when the Complaint was filed.

This evidence supports an inference of good-faith delay until facts of probable Hospital liability emerged, not mere suspicion or wishful thinking. (*Bockrath v. Aldrich Chem. Co.* (1999) 21 Cal.4th 71, 82.) The Court finds persuasive Plaintiff's argument that naming the Hospital earlier without expert support could raise Code of Civil Procedure section 128.7 or malicious prosecution concerns. (Code Civ. Proc., § 128.7; *Bockrath*, *supra*, at p. 82.)

The competing inferences preclude summary judgment on the section 474 issue. The Court does not weigh those inferences on summary judgment. Defendant has not shown as a matter of law that Plaintiff's claim against the Hospital is time-barred.

CONCLUSION

The Motion for Summary Judgment is Denied.